

Pre-Travel Health Advisory

Afghanistan · 7 days · Other

Departing 25 April 2026 (4 nights in Kabul)

Traveller: Magid Cleos

Low risk

COMPLETED

Apr 24, 2026

TRIP AT A GLANCE	
DURATION	7 days
PURPOSE	Religious pilgrimage
TRAVELLING	Alone
ACCOMMODATION	Staying with family or friends (rural setting)
INSURANCE	Travel insurance confirmed

ITINERARY-SPECIFIC GUIDANCE		
Trip type: ONE_WAY		
This is a one-way trip from Lagos, Nigeria to Kabul, Afghanistan, with 4 nights in Kabul beginning 25 April 2026. The traveller is staying in a rural setting with family or friends for a religious pilgrimage, with planned hiking, farm and animal contact, crowded event attendance, and altitude travel activities.		
ROUTE GUIDANCE BY STOP		
STOP	COUNTRY	GUIDANCE
Kabul	Afghanistan	Afghanistan presents a very high overall health and security risk. On arrival, do not travel after dark. Confirm your accommodation address and local emergency contacts before landing. Carry bottled water at all times and do not consume tap water or ice of unknown origin. Apply insect repellent immediately on arrival and maintain this throughout the stay, particularly during outdoor and rural activities. If you have not completed your rabies pre-exposure prophylaxis series before departure, be especially cautious around all animals including dogs, cats, and farm animals, and know the location of the nearest facility that can provide post-exposure treatment. Attend all planned religious and crowded events with heightened respiratory hygiene awareness; wear a well-fitting mask in densely crowded indoor settings. Follow all security advisories and avoid unnecessary movement outside your immediate area. Maintain daily contact with your emergency contact at home.

HEALTH RISK OVERVIEW		
CATEGORY	LEVEL	SUMMARY
Food and water safety	HIGH	Afghanistan has very high rates of waterborne and foodborne illness including typhoid, hepatitis A, cholera, and traveller diarrhoea. Rural accommodation with family or friends significantly elevates exposure risk compared to hotel settings. Strict food and water precautions are essential throughout the trip.

Vector-borne diseases	HIGH	Malaria transmission occurs in Afghanistan, particularly in rural and low-altitude areas from May through November. Kabul city itself carries lower but non-negligible risk. Dengue and leishmaniasis (a parasitic disease spread by sandfly bites) are also present. Comprehensive insect bite prevention is mandatory.
Respiratory infections	HIGH	Afghanistan has a high burden of tuberculosis (TB), one of the highest rates globally. Staying with local family or friends in a rural setting and attending crowded religious events significantly increases respiratory exposure risk. Influenza and COVID-19 circulate year-round. Respiratory hygiene is critical.
Environmental health (heat, sun, air quality)	MODERATE	Kabul in late April experiences moderate temperatures but UV radiation at altitude (approximately 1,800 metres above sea level) is stronger than at sea level. Air quality in Kabul is frequently poor due to vehicle emissions and dust. Travellers with respiratory conditions should take particular care, though none are flagged for this traveller.
Injuries and road traffic safety	HIGH	Afghanistan presents a very high risk of injury from road traffic accidents, landmines in rural and peri-urban areas, and ongoing security threats. Road conditions are poor and driving standards are unpredictable. The security situation in Afghanistan remains extremely volatile. Travellers should follow all Foreign Ministry and embassy advisories and avoid unnecessary movement.
Rabies and animal contact	HIGH	Afghanistan is rabies-endemic. This traveller has indicated planned farm and animal contact activities in a rural setting. Pre-exposure rabies prophylaxis is strongly recommended. Any animal bite or scratch must be treated as a medical emergency requiring immediate wound care and post-exposure treatment.
Blood-borne and sexual health	HIGH	This traveller has indicated anticipated new sexual partners, no barrier protection planned, and a prior history of sexually transmitted infections (STIs). HIV, hepatitis B, hepatitis C, and other STIs are present in Afghanistan. The combination of unprotected sex with new partners and an STI history places this traveller at elevated risk. Condom use is strongly recommended. Hepatitis B vaccination status should be confirmed.
Altitude-related illness	MODERATE	Kabul sits at approximately 1,800 metres above sea level. While this is below the 2,500-metre threshold for classic acute mountain sickness, this traveller has indicated planned altitude travel and hiking activities. If travel extends to higher-altitude regions of Afghanistan (which exceed 2,500 metres in many areas), acute mountain sickness becomes a genuine risk. Gradual ascent and awareness of symptoms are advised.

FLIGHT & JOURNEY HEALTH

VTE Risk Level: **LOW**

PREVENTION MEASURES

- Longest flight leg is 5 hours, which places this traveller in the medium-duration category; standard VTE (deep vein thrombosis) precautions apply
- Stay well hydrated throughout the flight; avoid excessive alcohol and caffeine, which promote dehydration
- Move around the cabin every 1 to 2 hours and perform seated leg exercises (ankle circles, calf raises) regularly

- No additional VTE risk factors have been identified for this traveller, so compression stockings are not mandatory but may be worn for comfort on the longer leg
- No MEDIF (medical information form for airline clearance) is required based on current health status

MEDICATION TIMING GUIDANCE

No insulin or time-sensitive medications are currently prescribed. If malaria prophylaxis (atovaquone-proguanil) is initiated before departure, take each dose at the same time daily regardless of time zone changes. Set a phone reminder to maintain consistency across time zones during transit.

VACCINATIONS

VACCINE	STATUS	GUIDANCE
Routine immunizations (e.g. MMR, varicella, dTdap, polio /IPV)	Believed up to date	Strongly recommended to verify and update before travel Afghanistan has active poliovirus circulation and is one of the few remaining polio-endemic countries. A polio booster (IPV) is strongly recommended regardless of prior vaccination history. Confirm tetanus-diphtheria-pertussis (Tdap) is current; a booster is recommended if more than 10 years have elapsed or if wound-prone activities such as hiking and farm work are planned. Confirm MMR (measles, mumps, rubella) two-dose series is documented. Measles outbreaks occur in Afghanistan.
Influenza	Unknown	Recommended Annual influenza vaccination is recommended for all international travellers. This traveller is attending crowded religious events and staying in close-contact household settings, both of which elevate respiratory transmission risk. Administer before departure if not received in the past 12 months.
COVID-19	Unknown	Recommended Ensure COVID-19 vaccination is up to date including any applicable boosters per current national schedule. Crowded pilgrimage and event settings carry elevated COVID-19 transmission risk. Check Afghanistan entry requirements at time of travel.
Hepatitis A	Not confirmed	Strongly recommended Hepatitis A is endemic in Afghanistan. Risk is significantly elevated by rural accommodation with family or friends and local food and water exposure. Two-dose series (0 and 6 to 12 months) provides long-term protection. A single dose administered at least 2 weeks before departure provides substantial short-term protection. Administer as soon as possible given the 25 April departure.
Hepatitis B	Not confirmed	Strongly recommended Hepatitis B is endemic in Afghanistan. This traveller has indicated anticipated new sexual partners without barrier protection and a prior STI history, which significantly elevates risk. An accelerated schedule (doses at day 0, day 7, and day 21 with a booster at 12 months) is available if time before departure is limited. Confirm immunity or initiate vaccination immediately.
Typhoid	Not confirmed	Strongly recommended

		Typhoid fever is endemic in Afghanistan with high transmission risk, particularly in rural settings with local food and water exposure. Either the injectable typhoid Vi polysaccharide vaccine (single dose, at least 2 weeks before travel) or the oral Ty21a vaccine (4 doses over 7 days, complete at least 1 week before travel) is appropriate. Discuss with your clinician which formulation is most suitable given the departure timeline.
Yellow fever	Not applicable	Not required or recommended Afghanistan is not a yellow fever endemic or risk country. Yellow fever vaccination is not required for entry and is not recommended for this destination.
Japanese encephalitis	Not applicable	Not routinely recommended; discuss with clinician Japanese encephalitis has limited documented risk in Afghanistan. Given the short duration of 7 days and urban-rural Kabul setting, routine vaccination is not indicated. If travel extends to rural agricultural areas with prolonged outdoor exposure during transmission season, discuss with your travel medicine clinician.
Meningococcal	Unknown	Recommended Meningococcal disease risk is elevated in Afghanistan, particularly in crowded settings. This traveller is attending crowded religious events, which is a recognised risk factor for meningococcal transmission. MenACWY vaccine is recommended. MenB vaccine may be considered; discuss with your clinician.
Rabies pre-exposure	Not received	Strongly recommended Afghanistan is rabies-endemic and this traveller has indicated planned farm and animal contact activities in a rural setting. Pre-exposure prophylaxis (PrEP) consists of 3 doses on days 0, 7, and 21 or 28. Given the 25 April departure, initiate immediately to allow completion before travel if possible. Even with PrEP, any animal bite or scratch requires immediate wound washing and urgent medical evaluation for post-exposure treatment. Access to post-exposure biologics in rural Afghanistan may be extremely limited, making PrEP especially important.
Cholera (oral vaccine)	Not received	Consider; discuss with clinician Cholera is present in Afghanistan. The oral cholera vaccine (Dukoral or Vaxchora depending on availability) provides partial protection against cholera and some protection against enterotoxigenic Escherichia coli (ETEC), a common cause of traveller diarrhoea. Given the high-risk food and water environment and rural accommodation, this is worth discussing with your travel medicine clinician, particularly if the departure timeline allows for the 2-dose schedule (doses 1 to 6 weeks apart for Dukoral).

MALARIA PREVENTION

Risk Level: MODERATE

Atovaquone-proguanil (Malarone) — preferred first-line agent for this traveller

Malaria transmission in Afghanistan is caused by both Plasmodium vivax (the

RECOMMENDED CHEMOPROPHYLAXIS	predominant species) and Plasmodium falciparum. Transmission occurs primarily from May through November in rural and low-altitude areas. Kabul city has lower but non-zero risk. This traveller is staying in a rural setting with family or friends, engaging in outdoor activities including hiking and farm work, and travelling in late April as the transmission season begins. Atovaquone-proguanil is recommended as the preferred agent: it is well-tolerated, has no neuropsychiatric side effects (relevant given this traveller's anticipated alcohol and substance use, which would make mefloquine unsuitable), and is effective against both Plasmodium species present. Doxycycline is an alternative but requires strict sun protection (photosensitising) and daily adherence. Mefloquine is not recommended due to this traveller's anticipated substance use and the neuropsychiatric risk associated with that combination. No contraindications to atovaquone-proguanil have been identified for this traveller. Clinician review and prescription are required.
MOSQUITO PROTECTION MEASURES	
▪ Apply DEET-based insect repellent (at least 30 to 50 percent concentration) to all exposed skin, reapplying every 3 to 4 hours and after sweating or swimming	
▪ Wear long-sleeved shirts and long trousers, particularly from dusk to dawn when Anopheles mosquitoes (malaria vectors) are most active	
▪ Sleep under a permethrin-treated bed net, especially in rural accommodation where window screens may be absent	
▪ Treat clothing and gear with permethrin spray for additional protection during outdoor and hiking activities	
▪ Avoid outdoor exposure during peak biting hours (dusk to dawn) where possible	
▪ Ensure accommodation is as well-sealed as possible; use a fan or air conditioning if available to reduce mosquito presence	
CONTRAINDICATIONS / ALTERNATIVES	Mefloquine is contraindicated for this traveller due to anticipated alcohol and recreational substance use (neuropsychiatric interaction risk). Primaquine and tafenoquine require G6PD (glucose-6-phosphate dehydrogenase) testing before use and are not first-line agents here. No contraindications to atovaquone-proguanil have been identified.

MEDICAL ADVICE & RECOMMENDATIONS	
TOPIC	DETAILS
Pre-travel review and vaccination records	Given the very short time before departure on 25 April 2026, arrange an urgent appointment with a travel medicine clinic or your primary care physician immediately. Bring all vaccination records. Priority vaccines to initiate or confirm are: hepatitis A, hepatitis B (accelerated schedule), typhoid, rabies pre-exposure prophylaxis, polio booster, meningococcal, and influenza. Some of these require multiple doses; the earlier you start, the better the protection before departure. Confirm your malaria prophylaxis prescription at the same appointment.
Food and water hygiene	In Afghanistan, assume all tap water, well water, and untreated water is unsafe. Drink only bottled water from sealed containers or water that has been boiled and cooled. Use safe water for brushing teeth. Avoid ice unless you are certain it was made from safe water. When staying with family or friends in a rural setting, politely but firmly maintain these precautions even if local hosts consider the water safe. Avoid raw or undercooked meat, fish, shellfish, and eggs. Avoid salads, raw vegetables, and fruit that cannot be

	peeled. Eat freshly cooked hot food where possible. Wash hands thoroughly with soap and water before eating and after using the toilet. Carry alcohol-based hand sanitiser (at least 60 percent alcohol) for situations where handwashing is not possible. If traveller diarrhoea develops, maintain hydration with oral rehydration salts. Discuss a standby antibiotic prescription (such as azithromycin) with your clinician before departure.
Vector bite prevention	Afghanistan has multiple vector-borne diseases including malaria, leishmaniasis (spread by sandfly bites, which are smaller than mosquitoes and can pass through standard mosquito nets), and dengue. Use a DEET-based repellent on all exposed skin at all times outdoors, not only at night. Sandflies are most active at dusk and dawn and in sandy or dusty environments. For leishmaniasis protection, use a fine-mesh bed net (with holes smaller than 0.6mm) or a permethrin-treated net. Wear light-coloured, loose-fitting, long-sleeved clothing. Check for ticks after hiking or farm activities and remove promptly with fine-tipped tweezers.
Sun, heat, and environmental precautions	Kabul is at approximately 1,800 metres elevation, where UV radiation is more intense than at sea level. Use SPF 50 or higher broad-spectrum sunscreen on all exposed skin, reapplying every 2 hours and after sweating. Wear a wide-brimmed hat and UV-protective sunglasses. Stay hydrated, particularly during outdoor hiking activities. Air quality in Kabul is frequently poor; if you experience respiratory irritation, limit outdoor exposure during high-pollution periods and consider a well-fitting mask. Avoid outdoor exertion during the hottest part of the day.
Injury and road safety	Afghanistan has extremely high rates of road traffic injury and fatality. Roads are poorly maintained, driving standards are unpredictable, and landmines remain a hazard in rural and peri-urban areas outside Kabul. Always wear a seatbelt. Avoid travel after dark. Do not travel to areas outside Kabul without reliable local guidance and security assessment. Follow all current Foreign Ministry travel advisories for Afghanistan, which at the time of this report indicate a very high security risk. Ensure your travel insurance includes emergency medical evacuation and security evacuation cover. Register your travel with your country's embassy in Afghanistan or the nearest embassy with consular responsibility.
Sexual health and blood exposure	This traveller has indicated anticipated new sexual partners, no planned barrier protection, and a prior history of STIs. HIV, hepatitis B, hepatitis C, syphilis, gonorrhoea, and other STIs are present in Afghanistan. Consistent and correct use of condoms with all new partners significantly reduces the risk of HIV, hepatitis B, and other STIs. Carry an adequate supply of condoms from home, as quality and availability in Afghanistan may be unreliable. Confirm hepatitis B vaccination before departure. Discuss PrEP (pre-exposure prophylaxis for HIV prevention) with your clinician before travel, particularly given the combination of new partners and no planned barrier protection. Avoid tattooing, piercing, or any medical or cosmetic procedures in Afghanistan unless in a verified sterile facility. If a needlestick injury or blood exposure occurs, seek medical attention immediately and ask your clinician about PEP (post-exposure prophylaxis for HIV) availability at your destination before you travel.
Jet lag, sleep, and mental wellbeing	Travelling alone to a high-risk, high-stress destination such as Afghanistan can be psychologically demanding. Ensure you have a clear communication plan with someone at home who knows your itinerary and can raise the

	alarm if contact is lost. Maintain regular sleep patterns where possible. Limit alcohol consumption, which impairs judgment, worsens dehydration, and interacts adversely with malaria prophylaxis and other medications. Be aware that culture shock, isolation, and the security environment can affect mental wellbeing. If you experience significant anxiety or distress, contact your primary care physician or a mental health support line.
Malaria and other chemoprophylaxis (state if not indicated)	Malaria prophylaxis is indicated for this trip. Atovaquone-proguanil (Malarone) is the recommended agent. Begin taking it 1 to 2 days before entering the malaria-risk area, continue daily throughout the trip, and for 7 days after leaving the risk area. Take each dose at the same time each day with food or a milky drink to improve absorption and reduce nausea. Do not skip doses. Carry the full course in your hand luggage. If you develop a fever during or after travel, seek medical attention immediately and inform the clinician of your travel to Afghanistan. Standby emergency treatment is not a substitute for prophylaxis but discuss with your clinician whether to carry a standby treatment course given the remoteness of your rural accommodation.
Traveller-specific considerations (from health context)	Several factors specific to this traveller require particular attention. First, this traveller is travelling alone to a very high-risk security environment; a robust emergency contact and communication plan is essential. Second, the combination of rural accommodation, farm and animal contact, and a rabies-endemic country makes rabies pre-exposure prophylaxis a high priority. Third, the anticipated substance use and alcohol consumption in the context of malaria prophylaxis (mefloquine is contraindicated; atovaquone-proguanil is preferred), sexual risk behaviours, and a demanding travel environment creates a compounding risk profile. Adherence to all medications and precautions is critical. Fourth, this traveller has no prior international travel in the last 12 months, making comprehensive pre-travel education and a travel medicine clinic visit especially important. Fifth, the religious pilgrimage purpose in a crowded setting elevates respiratory and meningococcal risk. Sixth, the traveller is departing from Lagos, Nigeria, and should ensure all Nigeria-specific health requirements (such as yellow fever vaccination certificate for re-entry to Nigeria on return) are also addressed.

MEDICATION LOGISTICS	
PACKAGING	Carry all medications in original manufacturer packaging with pharmacy labels intact. Carry a physician letter listing all prescribed medications by both brand and generic name, particularly for malaria prophylaxis and any controlled or injectable medications.
SUPPLY RULE	Carry a minimum of double the required supply, split between carry-on and checked luggage, to protect against loss or theft. Given the 7-day trip, carry at least 14 days of malaria prophylaxis supply.
DESTINATION LEGALITY VERIFIED	Yes — confirm all medications are legal at destination.

CLINICAL DECISION FLAGS
- TREE_3_MEDIUM_RISK_DESTINATION - TREE_4_DURATION

▪ TREE_5_FLIGHT
▪ TREE_9_ANIMAL_FARM_CONTACT_RABIES_ENDEMIC_DESTINATION
▪ TREE_9_CROWDED_EVENTS_MENINGOCOCCAL_RISK
▪ TREE_9_ALTITUDE_TRAVEL_FLAGGED
▪ TREE_13_NO_INTERNATIONAL_TRAVEL_LAST_12_MONTHS
▪ TREE_14_NEW_SEXUAL_PARTNERS_NO_BARRIER_PROTECTION
▪ TREE_14_STI_HISTORY
▪ TREE_14_SUBSTANCE_USE_ADHERENCE_RISK
▪ TREE_6_RELIGIOUS_PILGRIMAGE_ELEVATED_RISK
▪ TREE_8_RURAL_ACCOMMODATION_FAMILY_FRIENDS
▪ TREE_3_MALARIA_RISK_DESTINATION_AFGHANISTAN

CONTRAINDICATIONS (MEDICATIONS & VACCINES)
▪ Mefloquine is contraindicated due to anticipated alcohol and recreational substance use (neuropsychiatric adverse effect risk)
▪ No other active contraindications identified

SPECIALIST REFERRALS		
CONDITION	SPECIALIST	URGENCY
Comprehensive pre-travel risk profile including malaria prophylaxis, rabies pre-exposure prophylaxis, and multiple vaccine requirements with limited time before departure	Travel medicine specialist or infectious disease physician	URGENT
Anticipated unprotected sex with new partners and prior STI history; PrEP consideration	Sexual health clinician or primary care physician	BEFORE_TRAVEL
Substance use and alcohol anticipated; medication adherence risk and interaction with prophylaxis	Primary care physician	BEFORE_TRAVEL

SEXUAL HEALTH & RISK BEHAVIOURS
Risk Level: HIGH
PREVENTION ADVICE
▪ Use condoms consistently and correctly with all new sexual partners throughout the trip; carry an adequate supply from home as quality and availability in Afghanistan cannot be guaranteed
▪ Confirm hepatitis B vaccination status before departure and initiate accelerated schedule if not immune
▪ Discuss PrEP (pre-exposure prophylaxis) for HIV prevention with your clinician before travel given the combination of new partners and no planned barrier protection

Ask your clinician about PEP (post-exposure prophylaxis for HIV) availability at your destination before departure so you know where to access it if needed	
Avoid any tattooing, piercing, or cosmetic procedures in Afghanistan unless a verified sterile professional facility can be confirmed	
Arrange a post-travel STI screening appointment before departure so it is already scheduled for your return	
Be aware that alcohol and substance use impair judgment and increase the likelihood of unprotected sexual encounters; this is particularly relevant given the stated adherence risk	
PREP/PEP	PrEP/PEP availability discussed for high-risk destination.

AFTER YOU RETURN	
RED FLAGS — SEEK IMMEDIATE CARE IF YOU EXPERIENCE Seek emergency medical care immediately if you develop: fever at any point within 3 months of return (malaria emergency), severe headache with neck stiffness or rash (meningitis), difficulty breathing, chest pain, signs of severe dehydration, or any rapidly worsening symptoms. Always tell the treating clinician you have travelled to Afghanistan.	
WITHIN 1 WEEK	
Monitor for fever, chills, or flu-like symptoms; any fever within 3 months of return from a malaria-risk country must be treated as a medical emergency — attend an emergency department immediately and inform them of your travel to Afghanistan	
Monitor for signs of acute gastrointestinal illness including diarrhoea, vomiting, or abdominal cramps; seek medical attention if symptoms are severe or persistent	
Check for any skin lesions, rashes, or insect bite reactions that are not resolving normally	
WITHIN 4 WEEKS	
Attend a post-travel health check with your primary care physician or a travel medicine clinic, even if you feel well; many travel-acquired infections have delayed presentations	
Request post-travel STI screening including HIV, hepatitis B, syphilis, gonorrhoea, and chlamydia given the sexual risk behaviours during travel	
If you had any animal bite or scratch during the trip, seek immediate medical review regardless of whether you received pre-exposure rabies prophylaxis; post-exposure treatment may still be required	
Report any persistent fatigue, weight loss, night sweats, or lymph node swelling to your physician, as these may indicate TB exposure or other infections acquired in Afghanistan	
If you swam in or had contact with freshwater sources, mention this to your clinician for leptospirosis and schistosomiasis assessment	
BEYOND 4 WEEKS	
Some infections acquired in Afghanistan, including leishmaniasis and certain helminth (worm) infections, can present weeks to months after return; inform any treating clinician of your travel history for up to 12 months post-return	
Complete the hepatitis B vaccination series if an accelerated schedule was initiated before travel	
Complete the rabies pre-exposure prophylaxis series if not fully completed before departure	

If you received the hepatitis A single dose before travel, schedule the second dose 6 to 12 months later for long-term protection

MEDICAL CARE & CONTACTS	
CLINICS & FACILITIES	
NAME	LOCATION & NOTES
French Medical Institute for Mothers and Children (FMIC)	Aliabad, Kabul, Afghanistan +93 20 220 1234 · Central Kabul One of the most reputable hospitals in Kabul with international standards; verify current operational status and security situation before travel as conditions change rapidly
Aga Khan Health Service Afghanistan	Kabul, Afghanistan Verify locally on arrival · Kabul city Provides primary and secondary care; confirm current availability and access before travel
EMBASSY & CONSULAR	
NAME	DETAILS
Albanian Embassy (nearest with consular responsibility for Afghanistan)	Albania does not maintain a resident embassy in Afghanistan. Contact the Albanian Ministry of Foreign Affairs for consular assistance: +355 4 236 4090. Register your travel with the Albanian government travel registration service before departure.
Algerian Embassy in Kabul (current residence country)	Verify current operational status of the Algerian Embassy in Kabul before travel. Contact the Algerian Ministry of Foreign Affairs for guidance: +213 21 50 25 00.
EMERGENCY NUMBERS	
LABEL	CONTACT
Afghan Emergency Services	119 (police) / 102 (ambulance) — reliability is variable; do not rely solely on these
International SOS (if covered by travel insurance)	Confirm your travel insurance emergency assistance number before departure and save it in your phone
TMAG Emergency Reference	Always carry a printed copy of this advisory and your emergency contacts in case of phone loss or battery failure

NEXT STEPS
URGENT: Book an appointment with a travel medicine clinic or your primary care physician immediately given the 25 April 2026 departure date. Time is critical for initiating the rabies pre-exposure prophylaxis series (3 doses over 21 to 28 days) and the hepatitis B accelerated schedule. - Obtain prescriptions for: atovaquone-proguanil (malaria prophylaxis), standby antibiotic for traveller diarrhoea (discuss azithromycin with your clinician), and any other medications identified at your travel medicine consultation.

- Discuss PrEP for HIV prevention with your clinician or sexual health clinic before departure.
- Confirm or initiate: hepatitis A, hepatitis B (accelerated), typhoid, meningococcal MenACWY, polio booster (IPV), influenza, and COVID-19 booster vaccinations.
- Register your travel with the Albanian Ministry of Foreign Affairs and check the latest security and travel advisories for Afghanistan from your country's foreign ministry.
- Ensure your travel insurance policy includes emergency medical evacuation, security evacuation, and coverage for pre-existing conditions and high-risk destinations. Confirm the 24-hour emergency assistance number and save it in your phone.
- Prepare a travel health kit including: DEET repellent (50 percent), SPF 50 sunscreen, oral rehydration salts, water purification tablets or a portable filter, condoms, a digital thermometer, wound care supplies, and all prescribed medications in double supply.
- Inform a trusted contact at home of your full itinerary, accommodation address in Kabul, and a check-in schedule. Agree on what action they should take if contact is lost.
- On return, schedule a post-travel health check and STI screening appointment before you depart so it is already booked for when you return.

This advisory has been generated by the TMAG (Travel Medicine Advisory Global) AI Clinical Decision Support System and is intended to inform a pre-travel health consultation with a qualified clinician. It does not constitute medical advice and must not replace a consultation with a licensed physician, travel medicine specialist, or pharmacist. All vaccine and medication recommendations require clinician review and a valid prescription where applicable. Information is based on CDC and WHO travel health guidelines current at the time of generation; destination risk profiles may change. Always check for the latest alerts before departure. Travellers with complex medical histories, chronic conditions, or pregnancy are strongly advised to seek specialist travel medicine review in person before travelling. Medication guidance in this report is advisory only and must not be used to alter, substitute, or discontinue any medication currently prescribed to you. Please consult your physician or pharmacist before acting on any medication recommendation. Copyright 2026 Travel Medicine Advisory Global. travelmedicine.global

Travel Medicine Advisory Global — not a substitute for professional medical advice.

DOCTOR VERIFICATION & DIGITAL SIGNATURE

This travel health plan has been reviewed and approved by a licensed physician on the TMAG Doctor Network.

PHYSICIAN NAME

LICENSE NUMBER

MT898320084

VALIDATION DATE

24 April 2026

STATUS

TMAG Verified & Approved

Digital Signature

TMAG Verified Seal

This document was generated by Travel Medicine Advisory Global and approved by a licensed physician.